

****PATIENT DEMOGRAPHICS****

Name: Patient 43

Age: 55 years old | Sex: Male

MRN: 000-TEST-43

Admission Date: 06/12/2026 | Discharge Date: 06/15/2026

Attending Physician: Dr. M. Richardson, MD

****ADMITTING DIAGNOSIS & COMORBIDITIES****

Primary: Acute appendicitis with laparoscopic appendectomy

Secondary diagnoses: Type 2 diabetes mellitus (controlled), hyperlipidemia, mild postoperative ileus

****HOSPITAL COURSE****

Patient 43 presented via ED on 06/12/2026 with acute RLQ pain, fever (38.2°C), and elevated WBC 14.2K. CT imaging confirmed acute non-perforated appendicitis. Patient taken to OR for laparoscopic exploration and appendectomy performed without conversion to open procedure. Three trocars placed: 10mm umbilical (extraction site), 5mm at McBurney's point, 5mm at suprapubic location. Specimen extracted via endoscopic bag; hemostasis achieved with electrocautery. EBL minimal (~15mL). Total OR time 38 minutes.

Postoperative course uncomplicated. Patient extubated without incident; vitals stable throughout recovery. Trocar sites irrigated and closed with 4-0 absorbable sutures. Mild abdominal distention noted on POD#1, attributed to residual CO₂; resolved by POD#2 with ambulation and bowel function resumed. Patient tolerated advancement of diet per protocol:

- POD#0–1: NPO
- POD#1–2: Clear liquids, then soft diet
- POD#2+: Regular diet, well tolerated

Glucose monitoring q.i.d. maintained; home insulin regimen continued; fasting glucose 128–145 mg/dL (acceptable range). Pain managed with PO acetaminophen and NSAIDs; no opioid escalation required. Discharged POD#3 in stable condition, ambulating independently, tolerating full diet.

****DISCHARGE MEDICATIONS****

Medication	Dose	Frequency	Duration
-----	-----	-----	-----
Acetaminophen	650 mg	Q6H PRN pain	1 week
Ibuprofen	400 mg	Q8H PRN inflammation	1 week
Metformin	1000 mg	BID with meals	Chronic (resume)
Atorvastatin	20 mg	Q.d. at bedtime	Chronic (resume)
Insulin glargine	18 units	At bedtime	Chronic (resume)

****WOUND CARE & ACTIVITY****

All three trocar sites closed with absorbable sutures; no dressing required after 24 hours. Keep sites clean and dry. ****Do NOT submerge in water (bathing/swimming) for 5 days.**** Expect mild erythema and ecchymosis around port sites; this is normal.

****No lifting >10 lbs for 4 weeks.**** Avoid strenuous abdominal exercises, heavy gardening, and contact sports during this period. Light ambulation encouraged from discharge onward. Return to desk work in 1 week; manual labor in 3–4 weeks (per surgeon clearance).

****DIET****

Resume regular diet. Continue clear liquids x24 hours if any nausea or bloating. Avoid high-fat foods, spicy items, and excessive fiber for first 48 hours post-discharge to minimize GI irritation.

****FOLLOW-UP APPOINTMENTS****

1. ****General Surgery**** — Dr. M. Richardson, MD | ****In 2 weeks**